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Radiation Oncology

Medical Oncology

Surgical Oncology

DISCHARGE SUMMARY
DEPARTMENT OF RADIATION ONCOLOGY

Mr. K. NARASIMHA BHAT

73 Y / MALE

NOL 3-21-1847/L, VISHAKA, ALVARES ROAD, KADRI,
MANGALORE
Patient module no: 9448552461

Register Number: **PO21922**

IP Number: **IP00019629**

Admitted On: **06-08-2025 / 04:57 PM**

Discharged On: **13-08-2025 / 03:03 PM**

Ward Name: **PW-RAD**

Bed No: **RAD52**

PRIMARY CONSULTANT

Dr. Venkateraman Kini

RT NO: 546/25

AGNOSIS: High grade Urothelial Carcinoma, Muscle Invasive

INVESTIGATIONS:

Histopathology (tumour from left lateral wall of the urinary bladder) report on 13/09/2023:

- Papillary urothelial - high grade - muscle invasive - TURBT from right and left lateral walls.

MSI by IHC report on 26/12/2024: **Negative.**

PET CT scan report on 10/03/2025:

- Post TURBT status is seen with no evidence of residual active disease.
- Significant interval morphological regression of left level IV cervical, supraclavicular (0.9 x 0.5 cm) and right external iliac nodes with complete metabolic response.
- There is total regression of the previously noted retroperitoneal and common iliac group of nodes.
- The metastatic bone lesions now appear sclerotic with complete metabolic response. No new findings are seen.

PET CT scan report on 31/07/2025:

- Post TURBT status is seen with interval appearance of low grade FDG avid irregular wall thickening in the right lateral and posterior walls of urinary bladder - likely disease recurrence.
- Interval appearance of low grade FDG avid left external iliac node - likely metastatic.
- There is significant interval reduction in size of the left level IV cervical and supraclavicular nodes with resolution of metabolic activity.
- Prior noted non-FDG avid sclerotic bone lesions involving L3-L5 vertebral bodies show interval appearance of increased metabolic activity metabolic relapse.
- Overall findings suggest progressive disease.

TREATMENT GIVEN BEFORE REFERRAL: Received 4 cycles of Inj. GEMCITABINE + Inj. CARBOPLATIN, last cycle on 06/12/2024. Received 5 cycles of Inj. GEMCITABINE + Inj. CARBOPLATIN chemotherapy + Inj. ZOLEDRONIC ACID (last cycle on 16/04/2025)



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TREATMENT PLAN: Palliative Radiation Therapy to Lumbar Spine (3 DCRT)

TREATMENT GIVEN: Received Radiation 20 Gy/5# / 1 week to Lumbar Spine from 07/08/2025 to 13/08/2025 using 3 DCRT

COURSE OF TREATMENT: Patient tolerated radiation therapy well

POST TREATMENT ADVICE TO REFERRING DOCTOR/ GP:

- If patient complains of weakness/ Vomiting/ Nausea

Adv: Inj. Dexam 8mg + Inj. Rantac 50mg + Inj. Emetet 8mg in 500 ml NS IV over 2 hours - OD x 2/ 3/ alternate days

- If TC < 2000; Fever = 38°C ; loose stools; feeling unwell

Adv: IV antibiotics - Inj. Pipzo (Piperacillin + Tazobactam) 4.5 gm (8th hourly) and Inj. Amikacin 500mg IV BD (after test dose) x 5 days and inform the Oncologist

- In case of any emergency please contact (0824-4249999/8197807159)

FOLLOW UP APPOINTMENT: Review on 22/08/2025 with Dr. Hemant Kumar with investigation reports of CBC, Creatinine, RBS

- **PLAN:** Weekly Inj. NAB PACITAXEL + Inj. CARBOPLATIN Chemotherapy

Inj. SERPILULINAB


Dr. Venkataraman Kini M.D.
Radiation Oncology

Prepared by: (Signature)

Checked by: Dr. Hemant Kumar

Dr. VENKATARAMAN KINI
MD
Radiation Oncologist
Mangalore Institute of Oncology
Pumpwell, Mangalore - 575 002

KSHM/ 13-08-2025 02:03:26 PM



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Established in 1979
Approved by UGC, New Delhi, India
Since March 11, 1999

NAME	Mr. K NARASIMHA BHAT	STUDY DATE	31-07-2025 10:49:24
AGE	078Yrs	HOSPITAL NO	5213027
SEX	M	MODALITY	CT,PT
REPORTED ON	01-08-2025 12:56:11	REFERRED BY	Dr. HEMANTH KUMAR

F-18 FDG PET-CT REPORT

INDICATION:

High grade urothelial carcinoma-muscle invasive, post chemotherapy and TURBT.
Comparison is made with prior PET-CT scan dated 06.03.2025

STUDY DETAILS:

Radioisotope: 5.39mCi of F-18 FDG; SUVmax based on body weight

Fasting Blood Glucose: 112mg/dL

Ser Creatinine: 2.01mg/dL

Scanner: GE Discovery IQ Gen-2

Protocol: Non-Contrast CT scan (16-slice; 60-150 mAs; motion-free acquisition) was acquired from vertex to mid thigh followed by 1D-PET acquisition. Oral contrast was given.

HEAD & NECK:

Supratentorial:

Age related atrophic changes are seen in brain parenchyma. Small vessel ischemic changes are seen in periventricular white matter.

Paranasal sinuses and mastoid air cells are normal.

Bony calvarium appears normal. Orbital contents are unremarkable.

Neck:

Oral cavity and oropharynx - Nil significant.

Nasopharynx - Nil significant.

Hypopharynx - Nil significant.

Larynx - Nil significant.

Infratemporal fossa - Nil significant.

Masticator space - Nil significant.

Carotid space - Nil significant.

Pterygopalatine fossae - Nil significant.

Parapharyngeal space - Nil significant.

Retropharyngeal and prevertebral spaces - Nil significant.

Nasal cavity and paranasal sinuses - mucosal thickening is seen in right maxillary sinus.

Thyroid - Nil significant.

Lymph nodes - There is significant interval reduction in size of the left level IV cervical and supraclavicular nodes with resolution of metabolic activity, largest measuring 0.5x0.4cm, non FDG avid (previously measured 0.9 x 0.5 cm, non-FDG avid).

Visualised bones - Nil significant.



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NAME	MR. K. NARASIMHA BHAU	STUDY DATE	31.05.2024 10:48:24
AGE	67 Yrs	HOSPITAL NO	8218427
SEX	M	MODALITY	CT PT
REPORTED ON	31.05.2024 12:48:29	REFERRED BY	DR. SUDHANTH KUMAR

THORAX:

Lungs:

No focal mass lesions, consolidation, collapse, bronchiectasis or emphysematous changes noted.

Airway and Hilarum:

Trachea and major bronchi are normal.

No intraluminal filling defects present. No dilated bronchi seen.

Pleural Surfaces:

Bilateral apical pleural spaces appear normal.

No evidence of pleural effusion present.

Mediastinum:

Thoracic oesophagus and other mediastinal structures appear normal.

No significant mediastinal lymphadenopathy is observed.

Heart and Major Vessels:

Main pulmonary artery is normal.

No chamber enlargement noted.

No pericardial effusion noted.

Others:

Visualised bones appear normal.

Soft tissues and muscles of chest wall are normal.

ABDOMEN & PELVIS:

Liver: Normally positioned and has normal size and smooth borders. Its internal structure and attenuation values are normal. The intrahepatic bile ducts are normal.

Spleen: Normal in size and has smooth outer contours. No focal lesions noted.

GB and Biliary System: Normal in wall thickness. No calculi seen.

Pancreas: Normal in size. No calcification, duct dilatation or focal lesions noted.

Adrenals: Nil significant.

Kidneys: Normal size. No cyst/hydronephrosis. Calculi is seen in right kidney measuring 3mm in size.

Bowel, Mesentery, Omentum and Peritoneum: There is fat stranding in the mesentery - panniculitis.

Lymph Nodes: Prior noted non FDG avid right external iliac node remains stable in size measuring 0.8x0.8cm.

Interval appearance of low grade FDG avid left external iliac node is seen measuring 8mm in size.

Abdominal Vessels: Nil significant.

Fluid in Abdomen: Nil.



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AICWA No. 10/2018

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Urinary Bladder: Post TURBT status is seen. Interval appearance of low grade FDG avid irregular wall thickening is seen in the right lateral wall and posterior wall of urinary bladder with thickness of 8mm, SUV 2.5 (as seen on post laxix delayed images). No involvement of VU junction.

Prostate: Enlarged in size.

Inguinal Orifices: Nil significant.

Abdominal Wall: Nil significant.

Bones: Prior noted non-FDG avid sclerotic bone lesions involving L3-L5 vertebral bodies show interval appearance of increased metabolic activity, SUV 8.0 in L3 vertebra. Prior noted non-FDG avid lytic sclerotic lesion in left ischium remains stable.

IMPRESSION:

- Post TURBT status is seen with interval appearance of low grade FDG avid irregular wall thickening in the right lateral and posterior walls of urinary bladder - likely disease recurrence.
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- There is significant interval reduction in size of the left level IV cervical and supraclavicular nodes with resolution of metabolic activity.
- Prior noted non-FDG avid sclerotic bone lesions involving L3-L5 vertebral bodies show interval appearance of increased metabolic activity - metabolic relapse.
- Overall findings suggest progressive disease.

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